

Standard Operating Procedure

Nutritional Screening and Therapeutic Diet Management

«Hospital Name» — Supports COP.19.a through COP.19.d

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» screens every patient for nutritional risk, escalates at-risk patients to a dietician, and delivers the correct diet to the correct patient.

2. Scope

Applies to every admitted patient.

3. Responsibility

Doctor / Nurse (screener): screens on admission.

Dietician: assesses at-risk patients and plans the diet with the doctor and patient/relative.

4. Procedure

1. Screen every patient for nutritional risk on admission, using the hospital's adopted validated tool (e.g. the Malnutrition Universal Screening Tool).
2. For at-risk patients, a dietician — not the original screener — completes a full nutritional assessment covering planning, preparation, and distribution.
3. Plan the diet collaboratively: dietician, treating doctor, and patient/relative, accounting for allergies, food habits, and preferences.
4. Write a diet order for every patient, kept in a consistent location in the record. For therapeutic diets, confirm the diet sheet matches what is actually delivered.
5. Where family brings food, the dietician explains diet limitations and separately monitors what is actually brought against the prescribed diet.

5. Records

Supported by all 6 records in the COP.19 Required Records pack.

6. Related Documents

- HCO_COP_19_v2_REWRITE_DRAFT — COP.19 policy

7. Review

Reviewed annually.

8. References

- British Association for Parenteral and Enteral Nutrition (BAPEN). Malnutrition Universal Screening Tool ('MUST'). <https://www.bapen.org.uk/must-and-self-screening/introducing-must/> — a five-step, freely available screening tool the hospital may adopt as its example.